EHR AFRICA -- DRY RUN WORKFLOW GUIDE

Insured Patient Flow

Corporate / NHIF / Company Insurance | Patient pays CO-PAY only at cashier

Flow Type	Avg Duration	Starts At	Ends At	Billing	Payment
Insured / Corporate	**45-90 min**	**Gate / Insurance Desk**	**Pharmacy / Exit**	**Proforma + Co-pay**	**Patient pays CO-PAY at cashier**

PATIENT JOURNEY:

Gate > Insurance Verify > Registration > Triage > Consultation > Proforma > CO-PAY CASHIER > Pharmacy > Exit

STEP-BY-STEP PROCEDURE

1 Patient Arrives - Insurance Card Identified

[Gate Staff]

System: QMS > Gate Issue

- Gate staff identifies patient as insured when they show their insurance/company card.
- Issue QMS gate ticket - select patient type: Insured.
- Direct patient to Reception / Insurance Verification Desk.

2 Insurance Verification at Reception

[Receptionist / Medical Records]

System: Patients > Detail > Insurance Tab

- Ask patient for: Insurance card, Company ID, Authorization letter (if required).
- Retrieve or register patient in system.
- Open patient record, go to Insurance tab.
- Verify: Insurance company, policy number, coverage type, expiry date.
- Confirm ophthalmology / eye care services are covered.
- Note: Does insurance require Pre-Authorization before procedures?

3 Triage - Vitals & Chief Complaint

[OPD Nurse]

System: Consultations > Triage Workstation

- Record VA (right and left eye), IOP, blood pressure, chief complaint.
- Note any allergies.
- Submit triage - patient enters normal doctor queue.

4 Doctor Consultation

[Ophthalmologist / Doctor]

System: Consultations > Workstation

- Consultation follows standard clinical workflow.
- Doctor must be aware patient is insured - warn about uncovered services.
- If surgery or costly procedure recommended: Pre-Authorization may be required.
- Complete diagnosis, prescriptions, lab orders, investigation orders.
- Click Complete Consultation.

5 Pre-Authorization if Required

[Medical Records / Billing Officer]

System: Billing > Proforma OR Corporate Accounts

- For procedures (surgery, costly investigations): call/email insurer for pre-auth.
- Record pre-authorization number in system on the proforma.
- If not required: skip directly to Step 6.

6 Billing - Proforma / Insurance Claim

[Billing Officer / Cashier]

System: Billing > Proforma OR Billing > Cashier POS

- Open patient billing screen - charges auto-populated from consultation.
- Create Proforma Invoice addressed to insurance company / employer.
- Set billing tier: Insurance / Corporate Rate.
- SPLIT the bill: Insurance portion (billed to company on credit) + Co-pay (patient pays now).

7 CASHIER - PATIENT PAYS CO-PAY (MANDATORY STOP)

[Cashier]

System: Billing > Cashier POS

- **INSURED PATIENTS STILL PAY AT THE CASHIER - they pay the CO-PAY AMOUNT only.**
- Patient pays ONLY the co-payment amount (not the full bill).
- Insurance portion stays as outstanding on corporate account - claimed later by billing team.
- Process co-pay payment - receipt issued to patient.
- **PHARMACY WILL NOT RELEASE MEDICATION UNTIL CO-PAY IS CONFIRMED PAID.**

8 Pharmacy Dispensing

[Pharmacist]

System: Pharmacy > Dispensing Queue

- Prescription appears in queue ONLY after co-pay is confirmed.
- Dispense insurance-covered medications.
- Non-covered medications: patient pays full price separately.
- Mark as dispensed.

9 Patient Exit & Claim Filing

[Billing / Medical Records]

- Patient exits with medication and co-pay receipt.
- Billing officer consolidates claim at end of day/week and submits to insurer.
- Track reimbursements in Billing > Corporate Accounts > Statement.

NOTE: COVERAGE TIP: If unsure whether a service is covered, ask patient to confirm with their insurer before ordering. Document this in the consultation notes.

CHECKLIST: DRY RUN: (1) Insurance info recorded before consultation. (2) Proforma references correct corporate account. (3) Do NOT release medication until co-pay confirmed paid - even for insured patients.